

Physical examination

General assessment

It is recommended to evaluate the following aspects:

- **Extent of skin and mucous membrane lesions**, including the degree of mucosal damage and associated functional impairments such as dysphagia, dysphonia, weight loss, visual disturbances, and dyspareunia.
- **Disease activity quantification** using one of the two validated scoring systems: the **Pemphigus Disease Area Index (PDAI)** or the **Autoimmune Bullous Skin Intensity and Severity Score (ABSIS)** (4.5 ± 1.0) .

Assessment of the patient's general condition and co-morbidities should include:

- Body weight
- Arterial blood pressure
- Presence of co-morbidities (e.g., neoplastic, cardiovascular, musculoskeletal, diabetes)

The use of **DLQI**, and/or the **ABQOL** and **TABQOL** questionnaires—specific for autoimmune bullous diseases—may be considered (4.2 ± 1.2) .

Check vaccination status. Inquire about potential travel to countries where live vaccination is required; if so, ensure such vaccinations are completed before initiating immunosuppressive therapy.

The classical **Nikolsky's sign** is generally not recommended by most experts, as it may induce new lesions in already compromised skin and is not necessary for assessing disease activity given the availability of validated scoring tools.

Clinical subtypes

Pemphigus vulgaris (PV)

- Typically presents with **oral mucosal lesions**: painful, persistent erosions of the buccal mucosa and/or gingiva that interfere with eating.
- Less commonly, non-cicatricial lesions may affect the eyes, nose, larynx, esophagus, genitals, and anus.
- **Cutaneous involvement** may appear weeks to months after mucosal lesions. - Characterized by **flaccid bullae** with clear content, arising on non-erythematous skin, rapidly evolving into erosions. - Lesions may be localized or generalized, commonly affecting seborrhoeic areas (face, scalp, chest, interscapular region) and mechanically stressed areas, including extremities. - The disease is usually **not associated with significant pruritus**. - **Fingernail involvement** can occur.

Pemphigus foliaceus (PF)

- Presents with **cutaneous involvement only; no mucosal lesions**.
- Features transient, **flaccid bullae** or superficial, "puff pastry-like" exfoliation that progresses to crusted erosions.
- Predilection for **seborrhoeic areas**: scalp, face, chest, and interscapular region.
- More extensive disease is seen in sporadic and endemic forms (e.g., *Fogo Selvagem*, Brazilian pemphigus, Tunisian pemphigus).

Paraneoplastic pemphigus (PNP)

- Should be suspected in patients with an associated malignancy, particularly **non-Hodgkin's lymphoma, chronic lymphocytic leukemia, thymoma, or Castleman's disease**.

- The underlying tumor may not yet be diagnosed at the time of PNP presentation, and **PNP symptoms may precede malignancy detection.**

Mucosal involvement:

- Initial manifestations include **cheilitis** and/or **ulcerative stomatitis**, with persistent painful erosions leading to **severe dysphagia.**
- **Cicatricial conjunctivitis, keratitis, and genital involvement** are common.
- **Pharyngeal, nasal, and esophageal** involvement may occur, resulting in **phagodynia.**

Cutaneous involvement:

- **Polymorphic skin lesions** resembling: - Lichen planus - Graft-versus-host disease - Erythema multiforme - Bullous pemphigoid - Pemphigus vulgaris
- **Palmar involvement** is frequent.

Pulmonary involvement:

- **A characteristic and life-threatening complication, including:** - Alveolitis - Bronchiolitis obliterans - Pulmonary fibrosis